

OBSTETRIC CASE RECORD

Name : Mrs. Kalpara

OP / IP No.

Age : 26 Sex : F

Date of Admission :

Address : Royapuram

Date of discharge :

Ward No. :

Socio Economic Status :

Diagnosis : Education : 10th std.

Occupation : Housewife

Aadhar No. :

Mobile No. :

Obstetric Score : G₁P₁L₁

LMP : 1.3.21

EDD : 8.12.21

CHIEF COMPLAINTS:

The patient came with 9 months of amenorrhoea and has been referred from Royapuram PHC for raised blood pressure.

History of presenting illness : H/o lower abdominal pain on and off x 1 week. H/o leg swelling - 2 days Aggravated on standing, relieved on rest not associated with facial puffiness. No H/o diminished urine output, No H/o blurring of vision, No H/o epigastric pain, No H/o vomiting, headache. No H/o bleeding or draining PV.

Menstrual History:

Attained menarche at 13 years.

Regular cycle 4/28 days - Normal flow.

Not associated with pain/clots.

Marital History:

Married at 21 years of age.

Non-consanguineous marriage

No contraception used.

Gynaecologic History:

Previous pregnancy : conceived at 22 years of age; I, II, III trimester uncomplicated. NV at term in IOG; Male baby - delivered immediately after birth, No post partum haemorrhage, exclusively breast fed for 6 months. Baby is alive and healthy.

PRESENT PREGNANCY: Booked and Immunized at Royapuram PHC,
2 doses of Td given at 3rd, 5th month; Antenatal visits -

Past History:

I trimester: Pregnancy was confirmed by UPT at Royapuram PHC
4 weeks after missed period. H/o vomiting
No H/o fever with rash. No H/o radiation exposure
No H/o drug intake. No H/o bleeding PV. IFA tablets
taken. USG done at 3rd month.

II trimester: Quickening felt at 5th month, No H/o headache,
blurring of vision, abdominal pain, leg swelling.

Family History:

No H/o polyuria, polydipsia, polyphagia.
No H/o bleeding PV. USG done at 5th month.

III trimester: Able to perceive fetal movement, USG done
at 7th month. No H/o bleeding PV. Patient was
diagnosed to have hypertension before 2 weeks
and is on treatment.

Personal History:

PAST HISTORY: No H/o DM, HT, TB, Asthma, Epilepsy, Thyroid disease.
No H/o previous surgery
No H/o blood transfusion.

PERSONAL HISTORY: Mixed diet, Normal bowel and bladder habits.

Treatment History:

On drugs for pregnancy induced hypertension.

GENERAL PHYSICAL EXAMINATION:

Moderately built and nourished
No pallor, icterus.

Bilateral pitting pedal edema present upto knee.

CVS - S₁, S₂ heard, No murmur.

RS - Normal vesicular breath sounds heard, No added
sounds.

CNS - No focal neurological deficit

Breast, spine, thyroid, gait - Normal.

VITALS:

PULSE: 95/min BP: 130/80 mm Hg

RESP. RATE: 15/min

TEMP: Afebrile

Ht: 152 cm

Wt: 75 kg

BMI:

SYSTEMIC EXAMINATION:

INSPECTION: Abdomen longitudinally distended, umbilicus in midline.
Linea nigra, stria gravidarum seen.
Flanks full on sitting position.

PALPATION: Fundal height corresponds to 32 weeks of gestation.
Symphysio-fundal height - 32 cm.
Abdominal girth - 102 cm.

FUNDAL GRIP: Broad, soft, not independently ballotable part felt - corresponds to breech.

UMBILICAL GRIP: Rt. side - smooth curved hard uniform resistance felt - probably spine.
Lt. side - Multiple small nodules felt, probably limb buds.

I PELVIC GRIP: Hard, round, independently ballotable part felt - corresponds to head.

AUSCULTATION: FHS heard in Rt. spino-umbilical line.
Rate: 130/min, Regular rhythm.

DIAGNOSIS:

A 26 year old Mrs. Kalpana, G2, P, L1, whose LMP is 1.3.21 and EDD on 8.12.21, who has completed 38 weeks of gestation has single live fetus in cephalic presentation with good FHS has been diagnosed as a case of pregnancy induced hypertension and has been admitted for safe confinement of pregnancy.

INVESTIGATIONS:

Complete blood count
Urine - albumin, sugar
Blood urea, sugar
Serum creatinine
LFT, RFT, TFT
VDRL, HIV, HbsAg
Chest x ray, ECG, Echo.
USG Abdomen

TREATMENT:

DISCHARGE AND FOLLOW UP ADVICE